MEN’S HEALTH COMPASS

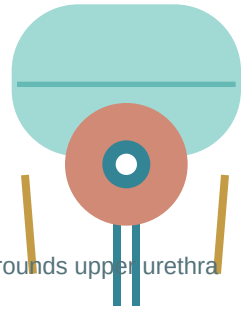
ILLUSTRATED RECOVERY SERIES

PROSTATE SURGERY RECOVERY COMPASS

A practical four-page guide to radical prostatectomy, recovery, function and life afterward.

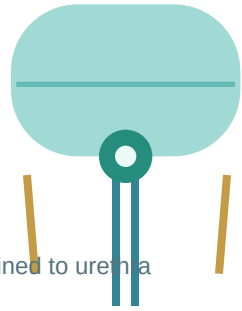
WHAT CHANGES DURING SURGERY

BEFORE



Prostate surrounds upper urethra

AFTER RADICAL PROSTATECTOMY



Bladder joined to urethra

Illustrative schematic, not to scale

THREE DIFFERENT RECOVERY TRACKS

PHYSICAL

Weeks: healing, mobility, energy

URINARY

Months: control may return gradually

SEXUAL

Months or longer; no guarantees

YOUR RECOVERY ROADMAP

Seven phases, not seven deadlines

01

BEFORE SURGERY

Confirm the procedure, medicines, home help, baseline urinary and sexual function, and who to call afterward.

02

SURGERY THROUGH ~2 WEEKS

Follow discharge instructions for catheter care, walking, rest, incisions, and bowel health.

03

AFTER CATHETER REMOVAL

Leakage may become noticeable. Discuss proper pelvic-floor technique and activity restrictions.

04

MONTHS 2-3

Build routines; review pathology and personalize PSA follow-up with your clinical team.

05

MONTHS 3-6

Assess urinary and sexual concerns, physical progress, and need for therapy or specialist care.

06

MONTHS 6-12

Persistent bothersome symptoms deserve evaluation; ask what treatment options may be appropriate.

07

YEAR 1 AND BEYOND

Continue cancer follow-up, manage ongoing effects, and support fitness, relationships and wellbeing.

WHEN TO GET HELP

Contact your surgical team for fever, catheter blockage, inability to urinate, worsening wounds or pain, or new one-sided leg swelling. Chest pain, sudden severe breathlessness or collapse need emergency care.

REGAINING FUNCTION

Continence and sexual health recover on different timelines

URINARY CONTROL

- **Understand the type**
  Stress leakage and urgency can require different care.
- **Train correctly**
  Ask when to begin pelvic-floor exercises and who can coach.
- **Track meaningful progress**
  Pad use, triggers and impact on daily activities.
- **Seek help if persistent**
  Discuss options at six months or a year when appropriate.

SEXUAL FUNCTION

- **Allow for uncertainty**
  Nerve sparing does not ensure unassisted erections return.
- **Know what changes**
  Orgasm may remain possible; semen ejaculation stops.
- **Discuss support options**
  Tablets, vacuum devices, injections or implants.
- **Protect intimacy**
  Relationships and emotional wellbeing also deserve care.

REHABILITATION AND EVERYDAY HEALTH

- Walk and rebuild strength at the pace your team recommends.
- Manage constipation, fatigue and sleep; follow activity restrictions.
- Favor balanced nutrition over unproven recovery supplements.
- Ask for support with stress, confidence, mood and relationships.

EVIDENCE BOUNDARY

No specific medication, device or pelvic-floor regimen guarantees full recovery. The right plan depends on the operation, baseline function and individual medical factors.

LIFE AFTER SURGERY

The follow-up plan should cover cancer control and quality of life

YOUR PERSONAL FOLLOW-UP CHECKLIST

PSA and pathology

Know the final pathology and who reviews each PSA result.

Urinary symptoms

Review leakage, urgency, pads used and treatment progress.

Sexual and emotional health

Discuss sexual options, intimacy, wellbeing and support.

Exercise and nutrition

Return to safe movement, balanced diet and general health.

QUESTIONS FOR YOUR NEXT VISIT

1. Which symptoms are expected for my procedure, and which need a call?
2. When should pelvic-floor or sexual-medicine referral happen?
3. If problems persist, which options should we evaluate?
4. Who orders PSA tests, and what changes the follow-up plan?

FURTHER READING

behshad.com/health/prostate/surgery-recovery/

AUA: Incontinence After Prostate Treatment (2024)

MSK: About Your Prostate Surgery (2026)

For educational use only. Not independently clinician peer reviewed. Your surgical team and discharge instructions take priority.